NOOR AALAM & 5 ORS v. PGIMER, CHANDIGARH VAKIL BRIEF

CC No. DC/44/CC/423/2026 · District Consumer Disputes Redressal Commission, Chandigarh
Next date: **22.12.2026 (oral arguments)** · Prepared on 30.10.2026 from the court record. The advocate should check facts and law before use. Separate papers: draft Written Arguments, expert brief, family summary.

1. The case on one page

Where the case stands

- Complainants: six legal heirs of Late Sh. Manohar, who died at PGIMER on 13.03.2026.
- Claim: Rs. 60,00,000 with 12% interest; Rs. 2,50,000 medical; Rs. 1,00,000 costs; duty doctors' details; preservation of records and CCTV.
- PGIMER denies negligence and asks for dismissal with exemplary costs.
- PGIMER's evidence closed (07.08.2026). Our rejoinder closed (14.10.2026). PGIMER's written arguments filed. **Our written arguments: 3 days from 14.10.2026 — filing not confirmed.**

What happened (PGIMER's own records unless marked)

- **11.03.2026** — Referred from GMSH-16 for ECG changes (Wellens). PGIMER's CCU sheet: "CAD – USA (Wellen's pattern)", anaemia, plan "correct anaemia"; Hb 7.5. CCU admission 10:10 am.
- **12.03.2026** — One unit of blood cross-matched and "kept ready for issue"; never issued. Moved out of the CCU onto a trolley; no ward bed. About 9:00 pm brought to the CCU once; pulse and oxygen checked; not re-admitted.
- **13.03.2026** — 12:15 am: "Received in CCU ... unresponsive & pulse was not palpable". Declared dead 12:51 am.

Five strongest points

1. PGIMER's own diagnosis; duty admitted — OP Annexure 3 (p.15); WS para 20-G.
2. Blood planned, kept ready, never given — OP Annexures 3, 6, 7; WS para 8.
3. Out of the CCU onto a trolley — admitted — WS paras 10, 11.
4. No recorded observation from 9:00 pm to 12:15 am — OP Annexure 5 (p.17).
5. Maintainable — limitation and jurisdiction admitted (WS paras 22–23).

Three main weak points (section 7)

1. No expert opinion yet; causation needs one.
2. PGIMER's letter: "no attendant came to collect" the blood.

3. Complaint errors to correct openly: "1:53 AM" (record: 12:51 am); "Complainant No.2" (the widow is No.3); "no monitoring after 8:30 AM".

Do now: confirm our written arguments; decide on an expert; decide on a records application.

Labels (timeline short form in brackets): PGIMER admits (Admitted: its written statement says so); Record shows (Record: a document says so); Entry (an entry exists; time or accuracy not proved); Family's account (Family: not yet proved); Does not match (two records differ); Open (needs a record, expert or family). "Our reading" = an inference to be confirmed.

2. Action list and contents

Task	Who	When	Why
Were our written arguments filed? (3 days from 14.10.2026: 16 or 17.10.2026). If not, decide the step.	Advocate	This week	Else only PGIMER's are on file
Get PGIMER's written arguments (14.10.2026) from the registry; answer new points.	Advocate	This week	Not in our papers
Decide on a cardiologist's written opinion (expert brief ready).	Advocate, family	Soon	Causation
Decide on a records application (section 10).	Advocate	Before 22.12	Night chart, transfer note
Check the notice service date in the court file.	Advocate	Before 22.12	45-day point
Check each authority against the official report (section 9).	Advocate	Before 22.12	None checked in a law report
Family answers (blood, bed, night, bills, income).	Family	1 week	Facts not in the record

Contents

1. The case on one page 1
2. Action list and contents 2
3. Documents map (page to cite) 4
4. Timeline, 10–13 March 2026 4
5.1 The complaint is maintainable 6
5.2 PGIMER's own diagnosis and admitted duty 6
5.3 Blood kept ready but never given 7
5.4 Out of the CCU onto a trolley 8
5.5 The last 3 hours 15 minutes 9
5.6 The chart against "monitored constantly" 9

5.7 Defences without documents 10
5.8 Cause of death on PGIMER's certificate 10
5.9 Causation and relief 11
6. The 45-day point (conditional) 11
7. Weak points and answers 12
8. Likely bench questions 12
9. Law and literature to verify 13
10. Records to ask PGIMER for 14
11. Glossary 14

3. Documents map (page to cite)

Complaint bundle: cite the court page (index number); the PDF copy runs two pages ahead. WS bundle: written statement pp.1–12, annexures pp.13–25.

Name	What it is	Filed by	Page to cite
Complaint	Complaint, memo of parties, prayer; affidavit of Complainant No.1	Family	pp.1–35
C-1 to C-5	GMSH-16 summaries (Feb–Mar 2026); lab reports incl. NT-proBNP; ultrasound	Family	pp.36–45
C-6	GMSH-16 referral summary (10–11.03.2026)	Family	p.46
C-7	GMSH-16 blood tests and ECGs (10.03.2026)	Family	pp.47–50
C-8	PGIMER emergency note and treatment sheet	Family	pp.51–54
C-9	PGIMER Emergency BP chart	Family	p.55
C-10	Blood reaction form, slip, Rs. 150 receipt	Family	pp.56–58
C-11	Death certificate (= OP Annexure 9)	Family	index p.59; copy marked "60" — check filed set
WS	Written statement (dated 26.06.2026, filed 01.07.2026)	PGIMER	pp.1–12
OP Ann-1	Point-of-care print: BNP 37.4; myoglobin 176, D-dimer 3280 flagged; no name or date	PGIMER	p.13
OP Ann-2, 3	CCU sheet: chest pain, ECG; "CAD – USA (Wellen's pattern)"; plan "correct anaemia"	PGIMER	pp.14–15
OP Ann-4, 5	Chest X-ray image; CCU vital chart and nurse's night note	PGIMER	pp.16, 17
OP Ann-6, 7	Hb 7.5 report; blood bank letter (01.06.2026) and register	PGIMER	pp.18, 19–20
OP Ann-8, 10	Drug chart; admission record (CCU 10:10 am; advance Rs. 2,000)	PGIMER	pp.21, 23
OP Ann-11, 12	Two ECG copies (no legible date, time or name)	PGIMER	pp.24–25
Orders	22.04, 29.05, 01.07, 07.08, 14.10.2026; e-Jagriti 22.10.2026	Commission	by date

4. Timeline, 10–13 March 2026

Every row of PGIMER's CCU chart (OP Annexure 5, p.17) is shown, with the gaps. "Entry" = time or accuracy not proved.

Date, time	What happened	Where	Kind
10.03, 1:07 pm	Hb 8.2 at GMSH-16	C-7, p.47	Record
11.03, 12:14 am	Referral to PGI Emergency: ECG " (Wellens)"; troponin negative	C-6, p.46	Record
11.03, 2:04 am	PGIMER emergency registration ("GENERAL")	C-8, p.51	Record
11.03	Emergency note: BP 130/100, pulse 116; "Imp - Wellens Syndrome"; shift to CCU	C-8, pp.52–53	Record
11.03, 5:00, 8:30 am	Emergency BP chart: 110/70; 120/88	C-9, p.55	Record
11.03, 9:30 am	CCU chart: BP 113/73, pulse 128, respiration 20	OP Ann-5	Entry
11.03, 10:10 am	CCU admission; advance Rs. 2,000	OP Ann-10	Record
11.03	CCU sheet "CAD – USA (Wellen's pattern)"; plan "correct anaemia"; Hb 7.5	OP Ann-3, 6	Record
11.03, 10:30 am	Chart row with dashes only	OP Ann-5	Entry
11.03, 12:00 pm	BP 11?/79 (digit unclear), pulse 125; 100°F, paracetamol	OP Ann-5	Entry
11.03, 3:00 pm	BP 124/103 (unclear), pulse 126, respiration 24	OP Ann-5	Entry
11.03, 6:00 pm	BP 90/49, pulse 106, respiration 26	OP Ann-5	Entry
11.03, 9:00 pm	BP 130/79, pulse 110, respiration 24	OP Ann-5	Entry
— 4 hours with no entry —			
12.03, 1:00 am	BP 110/74, pulse 100	OP Ann-5	Entry
— 8 hours with no entry —			
12.03, 9:00 am	BP 87/46; pulse "45" (unclear; could be 95)	OP Ann-5	Entry
12.03, 9:15 am	Blood request received at the blood storage centre	OP Ann-7	Record
12.03, 12:55 pm	Cross-match; unit "kept ready for issue"; bed "Trolly 66"	C-10, p.56; Ann-7	Record
— 4 hours with no entry (9:00 am to 1:00 pm) —			
12.03, 1:00 pm	BP 110/73, pulse 82, 37°C	OP Ann-5	Entry
12.03, 3:00 pm	BP about 93/60 (digit unclear), pulse 114; 37 or 38°C (unclear)	OP Ann-5	Entry
12.03, 4:47 pm	Rs. 150 paid for cross-matching	C-10, p.58	Record
12.03, ~5:10 pm	Out of the CCU onto a trolley (time: our reading of "nearly 31 hours")	WS PO-VI, 10–11	Admitted
— 6 hours with no entry (3:00 pm to 9:00 pm) —			

12.03, ~9:00 pm	Brought to CCU once: "mild breathlessness" (PGIMER) or chest pain (family); BP 90/60, pulse 82	WS 12, 14; Ann-5	Admitted
12.03, ~9:00 pm	Family: ACC blood bank found closed	Complaint 15	Family
— 3 hours 15 minutes with no entry (on a trolley, per PGIMER) —			
13.03, 12:15 am	"Received in CCU ... unresponsive & pulse was not palpable"; CPR	Ann-5; WS 18	Record
13.03, 12:51 am	Declared dead	Ann-5; C-11	Record

5. The arguments

5.1 The complaint is maintainable

Our point. PGIMER charged for this admission, admits limitation and territorial jurisdiction, and does not dispute that the complainants are consumers.

Proof. Advance Rs. 2,000 for the CCU admission — OP Annexure 10 (p.23). Rs. 150 for cross-matching, 4:47 pm on 12.03.2026 — C-10 (p.58). Limitation and territorial jurisdiction "admitted" — WS paras 22–23 (p.11). Filed on 07.04.2026, 25 days after the death.

PGIMER says jurisdiction fails only because "a provable deficiency of service does not exist" (WS para 2). **Answer:** that is the merits. Paid medical service is "service" (V.P. Shantha — check current status, section 9).

Say this in court

"PGIMER charged Rs. 2,000 for this admission and Rs. 150 for cross-matching. It admits limitation and territorial jurisdiction (WS paras 22–23) and nowhere disputes consumer status."

Do not say

That Rs. 2,50,000 was paid to PGIMER — no bills on record.

5.2 PGIMER's own diagnosis and admitted duty

Our point. PGIMER recorded Wellens and "CAD – USA (Wellen's pattern)", treated acute coronary syndrome, and admits a duty of care.

Proof.

- Referral "regarding his recent ECG findings [CAD ?ACS?] (Wellens)"; "(Wellens)" underlined, no question mark — C-6 (p.46).
- "Imp - Wellens Syndrome"; "Shift to CCU (ACC)" — C-8 (p.52). Chest pain since 11 pm on 10.03; ECG rate 123; new T changes V2–V4 — OP Ann-2 (p.14). "CAD – USA (Wellen's pattern)", "anaemia", plan "correct anaemia" — OP Ann-3 (p.15).
- ACS treatment (WS para 17, p.8); "The Opposite Party owed a duty of care to the deceased" (WS para 20-G, p.10).

PGIMER says the complaint "grossly exaggerates" (PO-III) and the GMSH ECG showed "only '?Wellens Syndrome' ... merely suspected" (WS para 3, p.4).

Our answer. (1) Does not match: the question mark is on ACS, not Wellens. (2) PGIMER's own records state Wellens without a query, and it treated ACS. (3) PGIMER's own print flags D-dimer 3280 (range 0–500) and myoglobin 176 (range 0–107) as abnormal (OP Ann-1); its CCU sheet copies the normal values but not these two (OP Ann-3); no follow-up is recorded. Meaning: for the expert.

Honest limits. Troponin negative, BNP 37.4, normal LVEF (C-6, C-8; OP Ann-1, 2). Argue Wellens/ACS and anaemia, not heart failure.

> **Say this in court**
> "PGIMER's own CCU sheet records 'CAD – USA (Wellen's pattern)' and a plan to 'correct anaemia'. It treated acute coronary syndrome and admits a duty of care. Its plea that Wellens was 'merely suspected' does not match the referral or its own records."

> **Do not say**
> "Heart failure"; "BNP ten times" (9.76 times; BNP and NT-proBNP are different tests).

5.3 Blood was kept ready but never given

Our point. PGIMER's plan was to "correct anaemia" and its report showed Hb 7.5 g/dL. A unit was cross-matched and kept ready. PGIMER admits it was never issued.

Proof.

- Plan "correct anaemia" — OP Ann-3 (p.15). Hb 7.5; same report's normal range 13.5–17.5 — OP Ann-6 (p.18).
- Request received 9:15 am on 12.03.2026; unit "kept ready for issue around 12 p.m." — OP Ann-7 (p.19).
- Cross-match 12:55 pm; transfusion boxes blank; printed rule "Get the Component issued only when the need for transfusion arises" — C-10 (p.56). Slip: "Kindly bring Blood Transport Box from the ward at the time of issue of blood" — C-10 (p.57). Rs. 150 paid at 4:47 pm — C-10 (p.58).
- No transfusion order in the drug chart — OP Ann-8 (p.21).

PGIMER says it was a "precautionary standby" for bleeding, Hb was "above 7 g/dL", "No medical indication" (PO-V); it was "never released or issued" (para 8); its doctor said there was "no immediate medical urgency", the trigger being "a further decline in the hemoglobin levels" (para 15).

Our answer. (1) "No indication" does not match PGIMER's own plan. (2) The blood bank kept the unit ready "for the patient". (3) PGIMER makes a further fall the trigger, yet no Hb after 7.5 appears in the record. (4) Its own form issues blood only when the need arises — the doctors' decision. (5) 7 g/dL is the general rule for stable patients;

guidance for heart patients uses higher levels [expert to confirm; section 9]. PGIMER filed no guideline.

Say this in court

"PGIMER's own plan was 'correct anaemia' and its own report shows Hb 7.5. A unit was cross-matched and kept ready, yet PGIMER admits it was never issued. Its own form issues blood only when the need arises — a decision for its doctors, who told the family there was no urgency."

Do not say

"Six hours to transfuse" (the six hours run from issue). "The family donated two units" (the slip's "Em 26 / 2 donations" is unexplained).

Weak spot. "No attendant came to collect" (OP Ann-7). **Answer:** letter written on 01.06.2026, after the complaint; PGIMER's doctor said no urgency; the box comes "from the ward at the time of issue".

5.4 Out of the CCU onto a trolley

Our point. PGIMER admits it moved him out of the CCU onto a trolley for want of a ward bed, and says the vital chart proves monitoring there. That chart has no entry after 9:00 pm.

Proof. WS para 11 (pp.6–7), also paras 10 and 20-b: "It is admitted that there was a temporary delay in the availability of a ward bed"; "Even on the trolley … monitoring … as conclusively evidenced by the vital chart". "nearly 31 hours" from 10:10 am (PO-VI) — our reading: about 5:10 pm on 12.03.2026. At 3:00 pm: pulse 114, BP about 93/60 (OP Ann-5). PGIMER's transfer rule: "coordinated between the Ward Senior Resident and the CCU Senior Resident, with the patient accompanied by hospital attendant or medical staff" (WS para 12); no transfer note or such record is produced.

Our answer. The shift and the trolley are admitted. We do not blame the shortage of beds; observation and an ECG needed no bed. The chart relied on stops at 9:00 pm.

Say this in court

"PGIMER admits it moved the patient out of the CCU onto a trolley because no ward bed was free. We do not blame the shortage of beds. But PGIMER says the vital chart proves he was monitored on the trolley — and that chart has no entry after 9:00 pm."

Do not say

That "Trolly 66" proves the evening trolley: the midday blood papers already show "Cardio CCU / Trolly 66" (C-10, pp.56–57). Open until the family answers and the bed register is seen.

5.5 The last 3 hours 15 minutes (night of 12/13 March)

On PGIMER's own papers: (1) out of the CCU, on a trolley (WS 10–11); (2) about 9:00 pm brought to the CCU once — "mild breathlessness" (PGIMER) or chest pain (family); pulse and oxygen checked, chest listened to; not re-admitted (WS 12, 14); (3) the chart's last entry is 9:00 pm: BP 90/60, pulse 82 — at 1:00 pm it was 110/73; (4) next: "Received in CCU at 12:15 ... unresponsive & pulse was not palpable"; dead at 12:51 am (Ann-5; WS 18).

A patient PGIMER itself diagnosed "CAD – USA (Wellen's pattern)" spent 3 hours 15 minutes on a trolley with no observation on record, and reached the CCU without a pulse.

- PGIMER gives the 9:00 pm account "to the best of our knowledge" and says saturation was "within normal limits" (WS 14). Its chart has no column for saturation; no value appears in any document produced.
- It says findings were "similar to those at initial admission" with "no tachycardia or tachypnoea" (WS 12); on the admission day its chart shows pulse 125–128 and respiration 24–26. Does not match.
- The written statement is silent on the 9:00 pm BP of 90/60. The night note mentions "ECG done" only at 12:51, with the death declaration.

Say this in court

"On PGIMER's own chart the last observation is 9:00 pm, BP 90/60. The next entry is 12:15 am: 'unresponsive and pulse was not palpable'. For a patient PGIMER had diagnosed with Wellen's pattern, no observation is recorded for three hours and fifteen minutes."

> **Do not say**
>
> "No monitoring at all" (say "none recorded"); "no ECG that night" (say "none of the 9:00 pm visit on record"); "no one came after 9:00 pm" (some evening drug-chart times are unclear).

Weak spot. The referral records "SOB x 5 days" before admission (C-6). Rely on the change on 12.03 evening, not on breathlessness alone.

5.6 The chart against "monitored constantly"

Our point. PGIMER pleads vitals "monitored constantly" and "hemodynamic stability throughout" (PO-VI; paras 15, 20-E). Its chart has 10 recorded readings over about 35 hours (the 10:30 am row is blank), including gaps of 4, 8, 4 and 6 hours, with BP 90/49 (6:00 pm 11.03), 87/46 (9:00 am 12.03) and 90/60 (9:00 pm 12.03), and pulse 125–128 on 11.03.

The drug chart's remark "vitals q_h" cannot be read on the copy (OP Ann-8). If the original reads 4- or 6-hourly, the 8-hour gap exceeds it (our reading; original needed).

Say this in court

"PGIMER's own chart has ten readings over about 35 hours, including gaps of four, eight, four and six hours, with BP readings of 90/49, 87/46 and 90/60. That is not 'monitored constantly'."

Do not say

"No monitoring after 8:30 AM" (complaint para 7). That came from the Emergency chart (C-9); withdraw it openly.

5.7 PGIMER's defences without documents

Our point. PGIMER's evidence was closed on 07.08.2026. Its medical defences rest on the written statement alone, and the documents it relies on are not produced.

- The written statement is verified "to the best of my knowledge and belief" by the Professor of Cardiology named as consultant on OP Ann-10 and C-11; no letter of authority (WS p.12).
- Not produced: committee comments (PO-II); "existing protocol" (paras 6, 10, 12); "guidelines" for Hb below 7 (paras 3, 8, 15, 20-D); counselling notes (PO-IV, 20-a); resuscitation record (para 19); transfer note; echo and X-ray reports. OP Ann-1, 11, 12 carry no legible date or name.
- Silent on prayers (iv) and (v): duty doctors' details; preservation of records and CCTV.

Say this in court

"PGIMER's evidence was closed on 07.08.2026. It relies on committee comments, protocols, guidelines and a resuscitation finding, but has filed none of them. Its own CCU sheet, chart and blood bank letter remain — and they support us."

Do not say

That PGIMER's annexures are worthless — we rely on them. No allegation of concealment or tampering.

5.8 Cause of death on PGIMER's own certificate

Proof. Part I: (a) Type I respiratory failure, (b) aspiration pneumonia, (c) coronary artery disease; Part II: pulmonary tuberculosis — C-11 = OP Ann-9 (p.22). Emergency note "Chest – B/L AE +" (C-8, p.52); at 9:00 pm chest findings "similar to when he was admitted" (WS 14).

PGIMER says death was "attributable entirely to ... these underlying conditions" (PO-IV); aspiration "is based on clinical findings observed during endotracheal intubation at the time of attempted resuscitation" (para 19).

Our answer. "Entirely" does not match a certificate that lists coronary artery disease in Part I. The aspiration finding comes from intubation during CPR; no resuscitation

record is filed. Whether aspiration pneumonia existed before the arrest is for the expert.

Honest limits. No post-mortem ("NOT WILLING FOR AUTOPSY", signed by Complainant No.1). Real earlier illnesses: TB, drug-induced hepatitis ("ATT Hepatitis" still listed on 10.03, C-6), sodium 124, fluid around both lungs (C-5).

Say this in court

"PGIMER's own certificate lists coronary artery disease in Part I. Its aspiration finding comes from intubation during resuscitation, and no resuscitation record has been filed."

Do not say

The certificate is false; the liver was normal; "1:53 AM".

5.9 Causation and relief

Causation. The sequence is PGIMER's own; whether the gaps caused or contributed to the death needs expert evidence. PGIMER says angiography was "deferred" on clinical grounds and the family told (para 20-a); no note is filed. Our reading (expert to confirm): if anaemia was a reason, correcting it — PGIMER's own plan — was part of the path. This answers para 20-a; it is not a new allegation.

Relief	Claimed	Proved by documents now
Compensation	Rs. 60,00,000 + 12%	Death and heirs; income not documented
Medical expenses	Rs. 2,50,000	Paid to PGIMER: Rs. 2,000 + Rs. 150 = Rs. 2,150
Litigation expenses	Rs. 1,00,000	—
Doctors' details; records, CCTV	Prayers (iv), (v)	PGIMER silent

PGIMER says the claim lacks "documentary evidence of income" (para 21), accepts three judgments as "settled law" but says they "have no application" (20-H), and asks for exemplary costs. **Answer:** the complaint rests on PGIMER's own records; keep every allegation within them.

Do not say

"Negligence proved" or "res ipsa loquitur" as settled; new facts outside the complaint (e.g. a shop sale) without the advocate's decision.

6. The 45-day point (conditional)

The order of 22.04.2026 gave "30 days from the date of receipt of notice extendable for 15 days (total 45 days)". The written statement, dated 26.06.2026, was filed on 01.07.2026. It is late only if notice was received by 16.05.2026 (or 17.05.2026,

depending on the counting rule); the date is not in our papers. On 07.08.2026 the complainants took time for a rejoinder with no objection recorded — PGIMER may call that waiver. **Raise it only if the service report supports it, after checking the provision and the law.**

7. Weak points and answers (open ones)

Weak point	Answer
No expert opinion	PGIMER has none either (evidence closed). Expert brief ready.
No post-mortem; "NOT WILLING FOR AUTOPSY" signed by Complainant No.1	Do not deny it. Rely on PGIMER's certificate (CAD in Part I); ask for the resuscitation record.
"No attendant came to collect" (OP Ann-7)	Written after the complaint; doctor said no urgency; issue only when the need arises (C-10).
Complaint errors: "1:53 AM"; "Complainant No.2"; "no monitoring after 8:30 AM"; "no BP" at night	Correct openly: 12:51 am; widow is No.3; argue the gaps; the 9:00 pm BP is recorded.
Old illnesses; breathlessness before admission (C-6)	Expert to separate their share; argue the evening change and the missing 9:00 pm record.
Rejoinder closed; our written arguments unconfirmed	Answer the written statement in the written arguments (draft ready).
Income and bills not proved	Collect documents; tie the claim to proof.
No written CCTV request on record	Rely on prayer (v) and PGIMER's silence.
Later family papers differ from the complaint	Stay with the sworn complaint unless the advocate decides otherwise.

8. Likely bench questions

- **Post-mortem?** No; "NOT WILLING FOR AUTOPSY" (C-11). [Reason: ____ to be filled]
- **Where and when did he die?** Received unresponsive in the CCU at 12:15 am; dead at 12:51 am; before that on a trolley (WS 10–11, 18).
- **Why was the blood not collected?** PGIMER's doctor said no urgency (WS 15); its form issues blood only on need (C-10). [Family: ____ to be filled]
- **Expert opinion?** Not yet; PGIMER has none either. [____ to be filled]
- **Why "1:53 AM"?** The copy was misread; we accept 12:51 am. [Confirm with the family first.]
- **When were your written arguments filed?** [____ to be filled]
- **Hospitals are crowded — what could doctors do?** We do not blame the shortage; observation and an ECG needed no bed.

Final arguments are not cross-examination. Show: the orders of 01.07 and 07.08.2026; OP Ann-5; WS paras 11, 12, 14, 15, 17, 20-a; OP Ann-3 and Ann-1; C-10 and C-11.

9. Law and literature to verify

None checked against an official law report for this brief. Correct citations found in secondary sources are given; confirm each before use.

Authority	For	Check
CPA 2019, ss.2(7), 2(11), 2(42), 34, 35, 38, 69	Consumer; service; jurisdiction; written version; limitation	Text in force; s.38 sub-section for services (secondary sources: s.38(3)(a); one government release: s.38(2)(a))
IMA v. V.P. Shantha, (1995) 6 SCC 651	Paid medical service is "service"	Reference declined 07.11.2024 (three judges); review and curative dismissed; a 2026 petition reported (notice, no stay) — check latest
Charan Singh v. Healing Touch Hospital, (2000) 7 SCC 668; AIR 2000 SC 3138	Compensation in consumer cases	Complaint says "7 SCC 68" — correct to 668
Sarla Verma v. DTC, (2009) 6 SCC 121	Just compensation; multiplier	Complaint's "III (2009) ACC 708" is a parallel report; use SCC
Maharaja Agrasen Hospital v. Rishabh Sharma (C.A. 6619/2016, 16.12.2019)	Medical-negligence compensation	Report citation; para "11.5.4"
Jacob Mathew (2005); Kusum Sharma (2010)	Standard of care	Citations, paragraphs
Savita Garg, (2004) 8 SCC 56; Malay Kumar Ganguly, (2009) 9 SCC 221; V. Kishan Rao, (2010) 5 SCC 513	Hospital's burden; when expert evidence is needed	Holdings
Paschim Banga, (1996) 4 SCC 37; Parmanand Katara, (1989) 4 SCC 286	Resources; emergency care	Use in a consumer case
New India Assurance v. Hilli Multipurpose (2020)	45-day limit	Citation; 2019 Act; waiver
Bharatiya Sakshya Adhiniyam 2023, s.63; admissions	Printouts; PGIMER's statements	Text; use before the Commission
Effect of closed evidence; of no specific denial	Weight of the written statement	Regulations, authority

AABB 2023 transfusion guideline	7 g/dL stable; 8 g/dL may be used with heart disease	Expert — literature
ACC/AHA 2025 ACS guideline	Hb at least 10 "may be reasonable" in ACS with anaemia (weak)	Expert
AABB 2025 guideline, myocardial infarction	Liberal threshold below 10 (conditional)	Expert; troponin here negative
StatPearls: Wellens syndrome	Normal troponin may mislead; urgent angiography	Expert — teaching review

10. Records to ask PGIMER for

Record	Why
Ward/trolley vital chart and nursing notes, 12.03 evening to 12:51 am	The 3 h 15 min
Transfer note; SR–SR coordination record; bed allotment register	Shift; "Trolly 66"
Resuscitation record and the 12:51 ECG	Aspiration; arrest
Original drug chart; all ECGs with machine time	"vitals q_h"; 9:00 pm
Duty roster and call book, night of 12.03	Prayer (iv)
Blood bank issue register, hours, requisition audit log	Blood
Committee comments; protocols; counselling notes; echo and X-ray reports; verifier's authority	PGIMER's pleas
CCTV of 12–13.03.2026 (preservation)	Prayer (v)
Registry: PGIMER's written arguments; service report	Answer; 45 days

11. Glossary

Term	Meaning
Wellens pattern	ECG T-wave changes in V2–V3 linked with a tight narrowing of a major heart artery (LAD).
ACS; CAD; USA	Acute coronary syndrome; coronary artery disease; "USA" = unstable angina (usual shorthand).
CCU	Coronary care unit (heart ICU).
Hb	Haemoglobin; PGIMER's normal range 13.5–17.5 g/dL; his 7.5.
Cross-match	Test matching a donor unit to the patient before transfusion.
Troponin; CK-MB	Markers of heart-muscle damage; here negative or normal.
BNP; NT-proBNP	Two different heart-strain tests; values not comparable.
D-dimer; myoglobin	Markers that rise with clots or muscle injury, and with other illness.

LVEF	Pumping strength of the heart; "(N)" = normal.
MCCD	Medical certificate of cause of death.
PTB; ATT	Pulmonary tuberculosis; anti-TB treatment (it can injure the liver).
Aspiration pneumonia	Lung infection after food or fluid enters the airway.
SR	Senior resident doctor.
WS; OP	PGIMER's written statement; opposite party (PGIMER).